Medicine Personal Statement: Selection Panel Feedback

UCAS three-question format • Reviewed 12 October 2026

Overall verdict

> **Rating: competitive but not yet outstanding - roughly 6.5/10 as it stands, with clear potential to reach 8+.** Total length is 3,995 of the 4,000 characters allowed, and each answer clears the 350-character minimum.

Answer	Characters	Words	Score	Assessment
Q1: Why medicine	623	101	5/10	Too short and too abstract
Q2: Studies and preparation	1,316	199	6/10	Strong science content, but doesn't fully answer the question
Q3: Experience outside education	2,056	299	6.5/10	Mature reflection, but leans on other people's stories

What would make me shortlist you: You reflect rather than list. You admit being wrong (the obesity exchange), you show real scientific depth (CTE and tau), and you end on a mature idea (blame culture vs learning culture). That puts you above most statements in the pile.

What would make me hesitate:

1. You never mention your A-levels.
2. Almost every insight in Q3 comes from a story someone told you, not from something you did.
3. Q1 doesn't yet convince me why you want **medicine** rather than a science degree.

Answer 1: Why medicine? (5/10)

Strengths

- Telling apart “what I don't know” from “what science doesn't know yet” is a mature idea for an opening.
- “Patients rarely match textbooks line for line” is the right instinct. It shows you see medicine as applied judgement, not memorisation.

Problems

1. **It's too short.** At 623 characters it's the thinnest answer, yet it's the one every assessor reads most carefully. Aim for 900-1,100.
2. **The opening anecdote doesn't help you.** “Doodling on my dad's work laptop after hours” is detail that doesn't serve your argument, and “curious child googles things” is a very common opening. If you keep it, cut it to one clause.
3. **There's a jump in logic.** You go from “unsolved questions fascinated me” to “what

drew me to medicine was seeing how that knowledge is applied" - but seeing it where, and when? An assessor will immediately think: "Unsolved questions point to research. Why not biomedical science?" You need one concrete moment that turned curiosity into wanting to treat patients. Your shadowing or the CTE project could provide it.

4. **"Restore a degree of patient freedom" is vague.** It's an interesting phrase but undefined. Freedom from what: pain, dependency, disability? Either explain it with an example (e.g. a patient regaining independence) or replace it.
5. **The realities of the career are missing.** There's nothing on teamwork, long-term commitment, uncertainty or patient relationships. Q1 should show you know what the job is, not just the science.

> **How to fix it:** Keep the curiosity theme, add one specific moment where you saw science applied to a real patient, and close with what kind of doctor that makes you want to be.

Answer 2: Qualifications and studies (6/10)

Strengths

- The CTE content is accurate and impressive. Perivascular p-tau at the depths of cortical sulci is indeed the defining lesion in the consensus criteria, as distinct from the pattern seen in Alzheimer's.
- "Clinical presentations rarely fit a rigid checklist" is a good insight, and you link it to patient care.
- The paragraph on presenting to mixed age groups (analogies, building up new information, props) is good evidence of communication skills.

Problems

1. **Your A-levels are missing entirely.** The question asks how your qualifications and studies prepared you. Name your subjects and give one specific example each, such as a Chemistry concept that explains a drug mechanism, or a Biology practical that taught precision. This is the biggest gap in the statement.
2. **Was the CTE research an EPQ?** If so, say so - "EPQ on…" signals independent, assessed research. If not, say how you did it: which sources, how long it took, whether you read primary literature.
3. **MMA belongs in Q3.** The sentence about pressure, progression and technique describes extracurricular skills. In Q2 it also has a grammar problem: "It has taught me…" follows a sentence about research, so "it" reads as the research, not MMA.
4. **You repeat yourself.** "My passion for mixed martial arts… a strong passion of mine." Cut the second.
5. **Your research is slightly out of date.** "CTE eludes definitive antemortem diagnosis" is true for the pathology, but since 2021 there have been clinical criteria for Traumatic Encephalopathy Syndrome (TES), and tau-PET imaging is under study. One clause on this would show you're following current research. At interview it's essential.

6. **There's a tension you don't mention.** You practise a sport linked to the disease you studied. An assessor will notice. Did the research change how you train, or what you think of consent and risk in contact sport? One reflective sentence here could be the most memorable line in the whole statement. Expect it at interview either way.

Answer 3: Outside education (6.5/10)

Strengths

- The anaesthetist exchange is the best moment in the statement. You were wrong, you were corrected, and you learned something about the biopsychosocial model. Assessors value this kind of humility.
- Moving from the consultant's error to Black Box Thinking to blame culture shows you understand patient safety, the duty of candour and learning from mistakes - core NHS values.
- The ending links back to Q1 (“childhood fascination with the unknown”), which is a nice structural touch.

Problems

1. **Too much is second-hand.** The porter's story, the consultant's story and the opioid conversations are all things you heard. Medical schools want evidence of what you did, saw and felt. Only the charity paragraph involves your own actions, and it's the vaguest one.
2. **The charity paragraph needs specifics.** What was your role at Rooms From The Heart? For how long, and how often? Describe one real situation with an aggressive resident: what you did and what you learned. “Wanting to help people has little value without action” is a platitude - show the action instead.
3. **The opioid sentence drifts.** “Regulatory flaws surrounding the international opioid crisis… commercial interests” reads as name-dropping and has no personal reflection. Either connect it to you (e.g. the consultant's opiate error shows prescribing safety mattering at both the individual and system level) or cut it and use the space for your own experience.
4. **“Countertransference” is risky jargon.** Strictly, it's a psychotherapy term for a clinician's emotional reaction to a particular patient. The porter's story is closer to identifying with the patient, or simply the emotional toll of resuscitation. An interviewer may ask you to define it. Plain English is safer, e.g. “seeing themselves in the patient”.
5. **“Emotional transparency” is unclear.** You probably mean debriefing, peer support and reflective practice - Schwartz Rounds, hot debriefs after arrests. Name them; it shows you understand how doctors actually cope.
6. **The obesity line needs care.** “Patients may use coping mechanisms for deep-seated personal trauma” could read as a generalisation. Phrase it as “for some patients, eating may be a coping mechanism” and note that obesity is multifactorial (genetics, environment, socioeconomic factors, medication). This avoids any hint of weight stigma.

7. **A minor confidentiality point.** A porter recounting a specific patient's arrest is fine as written (anonymised), but don't add identifying detail. Some interviewers may ask how you'd feel about staff sharing case stories.
8. **Some competencies are missing entirely.** There are no clear examples of: **teamwork or leadership** (MMA training partners, or anything at school); **sustained, direct contact with patients or service users** (care home, hospice, GP or hospital volunteering); **self-care and resilience** (MMA fits here naturally as how you manage stress).

Style and language

- **Over-elevated vocabulary.** “Tremendously”, “immense”, “profound”, “raw exposure”, “consolidated” pile up. Assessors read thousands of statements and notice thesaurus writing. Plain, precise English reads as more confident.
- **Good:** UK spelling is consistent (“realised”, “anaesthetist”) and sentence structure is generally controlled.
- **Typo in your file:** the Q2 heading repeats the Q1 question (“Why do you want to study this course…”). It doesn't matter on UCAS, but check your draft there.

Space budget

You're at the character limit, so every addition needs a cut. A suggested rebalance:

Answer	Now	Target	Cut	Add
Q1	623	~1,000	Laptop detail	A concrete trigger moment and what the job involves
Q2	1,316	~1,300	MMA skills sentence (move to Q3), repeated “passion”	A-level subjects, EPQ status, TES mention
Q3	2,056	~1,700	Opioid sentence, part of the porter paragraph	Specific charity incident, teamwork, MMA as resilience

Interview risk list

Anything in your statement is fair game at interview. Be ready to answer:

1. “You do MMA but researched CTE. How do you reconcile that?”
2. “What are the TES criteria? Can CTE be diagnosed in life now?”
3. “Define countertransference.”
4. “Tell me about one specific resident at Rooms From The Heart.”
5. “What does Black Box Thinking say about aviation versus healthcare? Do you know the Elaine Bromiley case?”
6. “What is the duty of candour?”
7. “What caused the opioid crisis?” (Purdue Pharma/OxyContin, marketing claims of low addiction risk, prescribing culture)

8. “Is obesity a personal responsibility or a societal problem?”
9. “Why medicine and not biomedical science?” (the gap in Q1)

Bottom line

The raw material is good: real intellectual curiosity, genuine reflection and a distinctive hook in MMA and CTE. The revision should:

- name your A-levels;
- put **your own actions** at the centre of Q3;
- give Q1 a concrete “why medicine, not science” moment;
- cut the jargon and the name-dropping.

> Do those and this becomes a statement that stands out. Rewrite in your own words - UCAS checks statements for similarity.